

1. Administrative & Study Identification

Full Study Title: A PHASE 3, MULTICENTER, DOUBLE-BLIND, RANDOMIZED, PLACEBO-CONTROLLED STUDY TO EVALUATE THE EFFICACY AND SAFETY OF PF-06823859 IN PARTICIPANTS WITH ACTIVE IDIOPATHIC INFLAMMATORY MYOPATHIES (INCLUDING PARTICIPANTS WITH ACTIVE DERMATOMYOSITIS OR POLYMYOSITIS) **Short Title/Acronym:** My(0)Path Trial / A Study to Evaluate PF-06823859 in People With Active Idiopathic Inflammatory Myopathies [Dermatomyositis (DM) and Polymyositis (PM)] **Protocol Number:** C0251006 **NCT Number:** NCT05895786 **Sponsor Name:** Pfizer Inc. **Investigational Product:** PF-06823859 (dazukibart) / anti-interferon beta therapy **Phase of Development:** Phase 3 **Study Status:** Recruiting **Last Update Posted:** 02-Apr-2026 **Medical Monitor:** Keith Kanik (Pfizer)

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the efficacy of PF-06823859 (dazukibart) in lessening muscle symptoms, weakness, and skin symptoms in adult participants with active dermatomyositis (DM) and adult participants with active polymyositis (PM). **Secondary Objectives:** To evaluate the safety, tolerability, and broader clinical outcomes of PF-06823859, including its effect on manual muscle testing (MMT-8), skin disease severity (CDASI-A), physical function (PROMIS-PF), and fatigue (FACIT-F).

2.2 Background & Rationale Idiopathic inflammatory myopathies (DM and PM) are rare, chronic autoimmune disorders that cause inflammation weakening the muscles important for movement and potentially causing skin rashes. The study medicine, dazukibart, works by blocking an immune system protein called interferon β (IFN β). IFN β is found in higher levels in people with DM and PM and is thought to drive the progression of these diseases by causing debilitating inflammation in the skin and muscles.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Primary Purpose:** Treatment **Control Type:** Placebo-controlled **Intervention Model:** Parallel Assignment **Blinding:** Quadruple-blind (Participant, Care Provider, Investigator, Outcomes Assessor) **Randomization:** Randomized (2:1 allocation to dazukibart or placebo)

3.2 Planned Enrollment & Duration Target **\$N\$**: Approximately 270 participants across 2 parallel cohorts (Cohort 1 [DM]: **\$N=111\$**; Cohort 2 [PM]: **\$N=159\$**) **Number of Sites**: Multicenter (Global, including US) **Estimated Study Start**: 20-May-2023 **Estimated Primary Completion**: 28-Jul-2027 **Estimated Study Completion**: 28-Jul-2027

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Age ≥ 18 years. 2. Sex: All (Male and Female). Healthy Volunteers: No (Not accepted). 3. Confirmed diagnosis of active dermatomyositis (DM) or active polymyositis (PM) with muscle weakness. 4. Receiving a stable dose of 1 corticosteroid taken by mouth and/or 1 traditional immunosuppressant as standard of care background medication.

4.2 Exclusion Criteria 1. Any other type of idiopathic inflammatory myopathy or myositis caused by other reasons. 2. Current bacterial, viral, fungal, mycobacterial, or other infections. 3. Clinically significant finding on a chest X-ray in the last 12 weeks, cancer within the last 5 years, or prior treatment with any type of anti-IFN β therapy.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Administered every 4 weeks. **Route of Administration**: Intravenous (IV) infusion (taking approximately 1 hour per infusion). **Duration of Treatment**: 48 weeks of active treatment (with the study encompassing a 52-week total observation period/13 months total duration).

5.2 Concomitant & Prohibited Therapy Permitted: Stable doses of oral corticosteroids and/or traditional immunosuppressants. **Prohibited**: Any other investigational product within 30 days prior to first dose, any prior anti-IFN β therapies.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Up to Day 1	Informed Consent, Medical History, Confirmation of DM/PM diagnosis, Chest X-ray
Baseline	Day 1	Randomization (2:1), First IV Infusion, Clinical scoring baseline assessments
Interim	Weeks 4 to 48	IV infusions every 4 weeks. Safety monitoring, TIS evaluation, MMT-8, CDASI-A, PROMIS-PF
End of Study	Week 52	Final observation assessments, TIS evaluation (US cohort primary endpoint), Exit Interview

(Note: Participants complete approximately 15 study visits across the 13-month participation).

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Moderate change in Total Improvement Score (TIS). **Measurement Method:** Total Improvement Score on a scale from 0 to 100 (higher scores indicate a better outcome), assessed at 24 weeks for participants outside of the United States (US) and at 52 weeks for participants within the US.

7.2 Secondary & Exploratory Endpoints * Change from baseline in Manual Muscle Testing - 8 designated muscles (MMT-8) at 24/52 weeks. * Change from baseline in Cutaneous Dermatomyositis Disease Area and Severity Index Activity Score (CDASI-A) at 24 weeks (in DM participants). * Standardized AUC for corticosteroid doses over 52 weeks.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard collection via onsite clinical visits every 4 weeks during IV infusions. **Serious Adverse Events (SAEs):** Specific monitoring for serious bacterial, viral, fungal, and mycobacterial infections due to the immunomodulatory nature of the investigational product. **Data Monitoring Committee (DMC):** Internal safety monitoring mechanisms managed by Pfizer Inc.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for efficacy outcomes (TIS, MMT-8). Safety Set for AE/SAE evaluations. **Sample Size Justification:** $N=270$ (Cohort 1: 111, Cohort 2: 159) appropriately powered to detect significant differences in the moderate change of Total Improvement Score (TIS) between the active drug and placebo. **Handling of Missing Data:** Standard multiple imputation or mixed models for repeated measures (MMRM) applied to clinical scoring parameters.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Continuous monitoring across 15 standard scheduled onsite visits. **Audit Trail:** eCRF locking, blinding compliance tracking, and data clarification forms via standard quality audit mechanisms.

11. Study Identifiers & Governance

Primary ID: C0251006 **Registry Number:** NCT05895786 **IRAS ID:** 1008220 **Sponsor/Lead Organization:** Pfizer Inc. **Study Title:** My(0)Path Trial / PF-06823859 in Active Idiopathic Inflammatory Myopathies **Official Regulatory Title:** A PHASE 3, MULTICENTER, DOUBLE-BLIND, RANDOMIZED, PLACEBO-CONTROLLED STUDY TO EVALUATE THE EFFICACY AND SAFETY OF PF-06823859 IN PARTICIPANTS WITH ACTIVE IDIOPATHIC INFLAMMATORY MYOPATHIES (INCLUDING PARTICIPANTS WITH ACTIVE DERMATOMYOSITIS OR POLYMYOSITIS)

12. Clinical Framework (Myopathy Specific)

Primary Condition: Idiopathic Inflammatory Myopathies (Dermatomyositis and Polymyositis) **Diagnosis Threshold:** Clinically active DM or PM with muscle weakness and characteristic symptomatology **Medical Methodology:** Standard of care utilization (traditional immunosuppressants and oral corticosteroids) combined with investigational anti-IFN β therapy **Optimization Target:** Reduction in IFN β -driven inflammation to lessen muscle/skin symptoms

13. Study Procedures & Methodology

Management Pathway: Standard clinical pathway for DM/PM maintaining stable background SOC treatment **Education Protocol (HCP):** Investigator protocol training & blinding requirement education **Education Protocol (Patient):** Onsite informed consent detailing 2:1 placebo probability and side-effect expectations **Quality Audit Mechanism:** Ongoing drug-accountability monitoring and adherence tracking for concurrent background therapies

14. Observation & Follow-Up Schedule

Total Duration: 52 Follow-up weeks (13 Months) **Onsite Visit Cadence:** Every 4 weeks (Q4W) mapping to 15 total visits **Remote Monitoring Frequency:** Routine oversight mechanisms mapped to AE reporting **Checklist Review Interval:** Every 4 weeks alongside IV infusions

15. Sign-off & Approval

Role	Name	Signature	Date	:---	:---	:---	:---
Principal Investigator	Alisa N Femia (NYU Site PI) / Global PI	_____	[DD-MMM-YYYY]	Clinical Operations Lead	Keith Kanik	_____	[DD-MMM-YYYY]
Lead Statistician	[Pfizer Sponsor Statistician]	_____	[DD-MMM-YYYY]				